03
Dashboard

The dashboard turns aid cuts into people losing services

01	**Question**	When donor health aid is cut in 2026-2030, how many people a year lose HIV, TB, malaria and routine immunization services in each of 70 countries, and how much of the lost aid can each government replace?
02	**Purpose**	For WHO and ministries of health: run any Donor Scenario country by country, see which services and channels lose most, and test each Government Response option against the country's fiscal space.

Any donor, channel, cut pace and replacement, for 71 countries

You set	Options, as labelled in the dashboard
Who cuts	Custom: the United States or any of 13 other donor groups, or every donor's money through the Global Fund, Gavi, WHO or UNICEF
How the cut flows	Per donor: Bilateral & NGO Cut % and Via Multilaterals Cut %; cuts combine as 1 − (1 − donor cut)(1 − channel cut)
How fast	How Funding Changes, 2026-2030: Sudden (Default), Linear Phase-Out, Front-Loaded, Back-Loaded, S-Curve, Custom
What fills the gap	Government Response: No Government Replacement (default), Replace a Set Share, As Much as Fiscal Space Allows or Gradual Budget Increase
Presets	Donor Scenario: US Bilateral Exit + Multilateral Withdrawals; America First MOUs; IHME 2025 Preliminary Estimates; Total-Aid Cuts (OECD) Applied to Health Aid; Global Fund & Gavi Shortfalls; Combined Retreat
Uncertainty	Range Across Parameter Values: 2.5th to 97.5th percentile of 400 parameter draws, beside every central estimate

Source: dashboard code (scenarios.py, model_section.py, paths_ui.py), October 2026; 71 countries listed, 70 with recorded aid (Panama has none).

Section A: How the dashboard is built

01	**Problem**	A cut is announced in US dollars; how many people then lose a service, and how much coverage falls, is not.
02	**Approach**	IHME aid by donor, channel and service, joined to cost per person served, people in need and government budgets for 70 countries, in five linked steps.
03	**Finding**	Any Donor Scenario runs from US$ lost to people losing each service and coverage points lost, with a 2.5th to 97.5th percentile range from 400 draws.

Five linked steps turn a donor's cut into lost services and coverage

1

Aid Money

By donor, channel, service

IHME DAH 1990-2026, 2021-23 average

2

Scenario Cut

Share cut, 2026-2030 path

KFF MOU tracker; OECD ODA 2025; IHME 2025

3

Government Response

Four replacement options

IHME Health Spending; WB WDI; IMF WEO

4

Services Lost

Net loss ÷ cost per person

GHCC unit costs (ART, TB), 2023 US$

5

Coverage Change

Lost ÷ people in need

UNAIDS, WHO, MAP, WUENIC, 2024-25

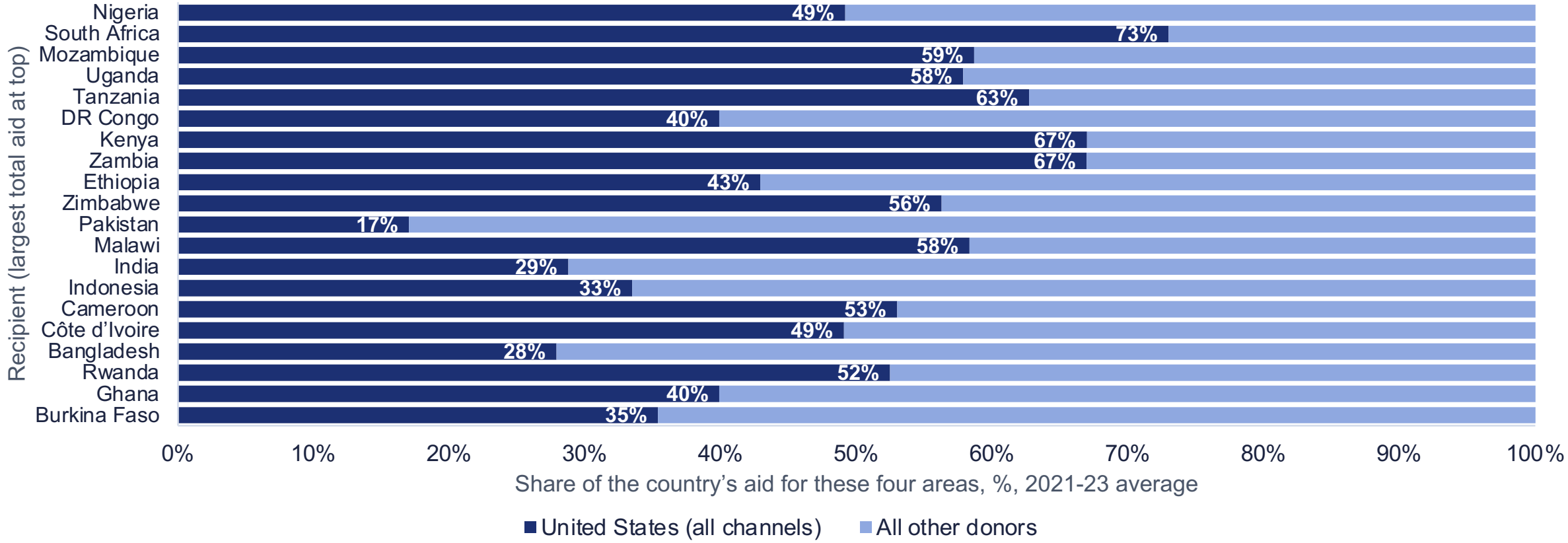
Sources: IHME DAH; KFF; OECD; IHME Health Spending; WB WDI; IMF WEO; GHCC; UNAIDS; WHO GTB; MAP; WUENIC (see References); health_model.py.

Section B: Where the money comes from

01	**Problem**	How exposed a country is to a cut depends on who pays for its services and through which channel.
02	**Approach**	IHME DAH aid for HIV, TB, malaria and immunization, 2021-23 average, split by donor, channel and IHME programme area for 70 countries.
03	**Finding**	The US pays US$5.2B of US$10.3B a year (50%), and over 50% in 10 of the 20 largest recipients.

The US pays over half the aid in 10 of the 20 largest recipients

US share of 2021-23 aid for HIV, TB, malaria and immunization, 20 largest recipients of the 70



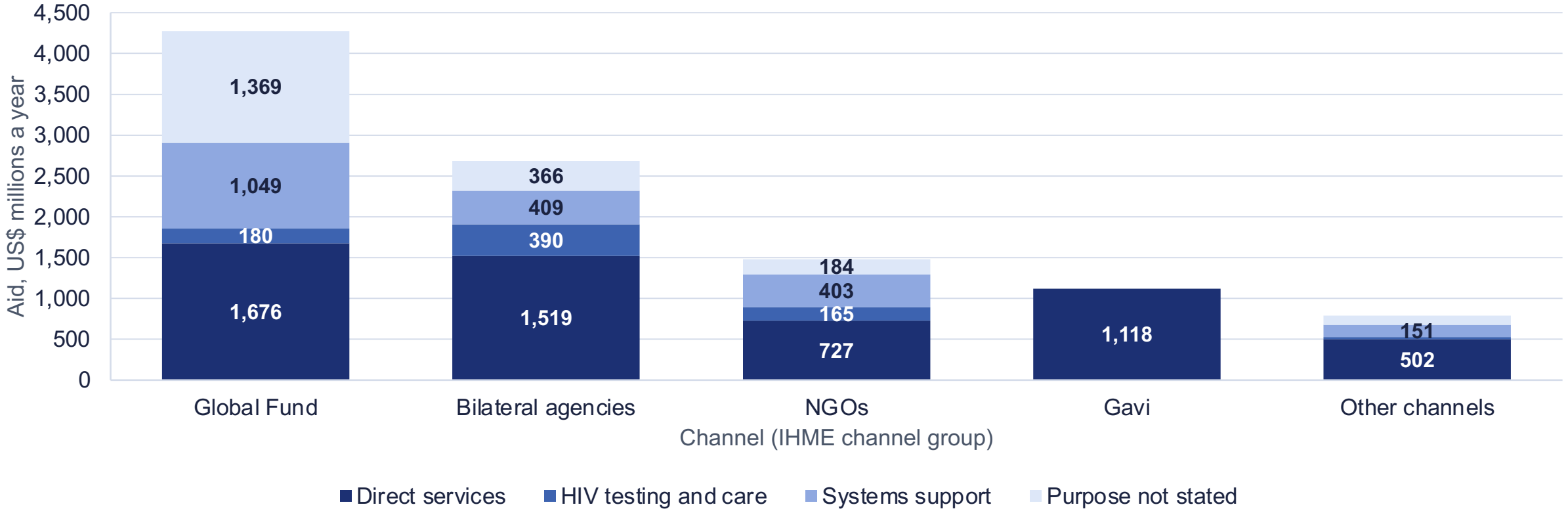
Across all 70 countries the US pays 50%: US$5.2B of US$10.3B a year, 2021-23 average, constant 2023 US$.

Source: IHME DAH 1990-2026, 2021-23 average, constant 2023 US$; model_data/dah_lines.csv (prepare_model.py).

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54% of aid pays directly for services; a fifth has no stated purpose

Aid by channel and IHME programme area, 70 countries, 2021-23 average



Direct services: ART, PMTCT, HIV prevention, OVC, TB care, bednets, spraying, malaria care, vaccines.

Of US$10.3B a year: systems support 19%, purpose not stated 20%, HIV testing and care 7%.

Source: IHME DAH 1990-2026, 2021-23 average, constant 2023 US$; HIV, TB, malaria and immunization areas grouped as in prepare_model.LINES.

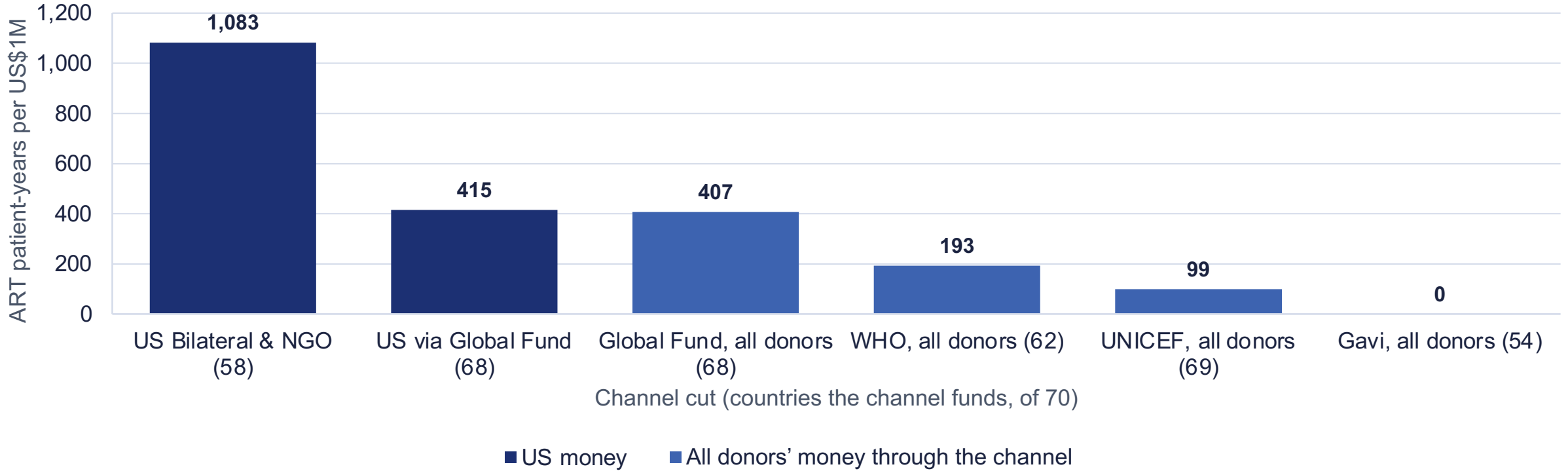
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Section C: What a cut removes

01	**Problem**	The same US$1M cut removes different services depending on the channel it flows through and the country.
02	**Approach**	Services Lost = net aid lost on each service ÷ cost per person served, capped at the people donor money pays for; the same for any donor and any channel.
03	**Finding**	On signed MOU schedules, ART coverage falls below 90% in two countries in 2026. US Bilateral Exit + Multilateral Withdrawals, the upper bound, stops ART for 4.86M people a year.

Per US$1M, US bilateral cuts remove 2.7× the ART of Global Fund cuts

ART patient-years lost a year per US$1M of aid lost a year, by channel cut, 2026-30



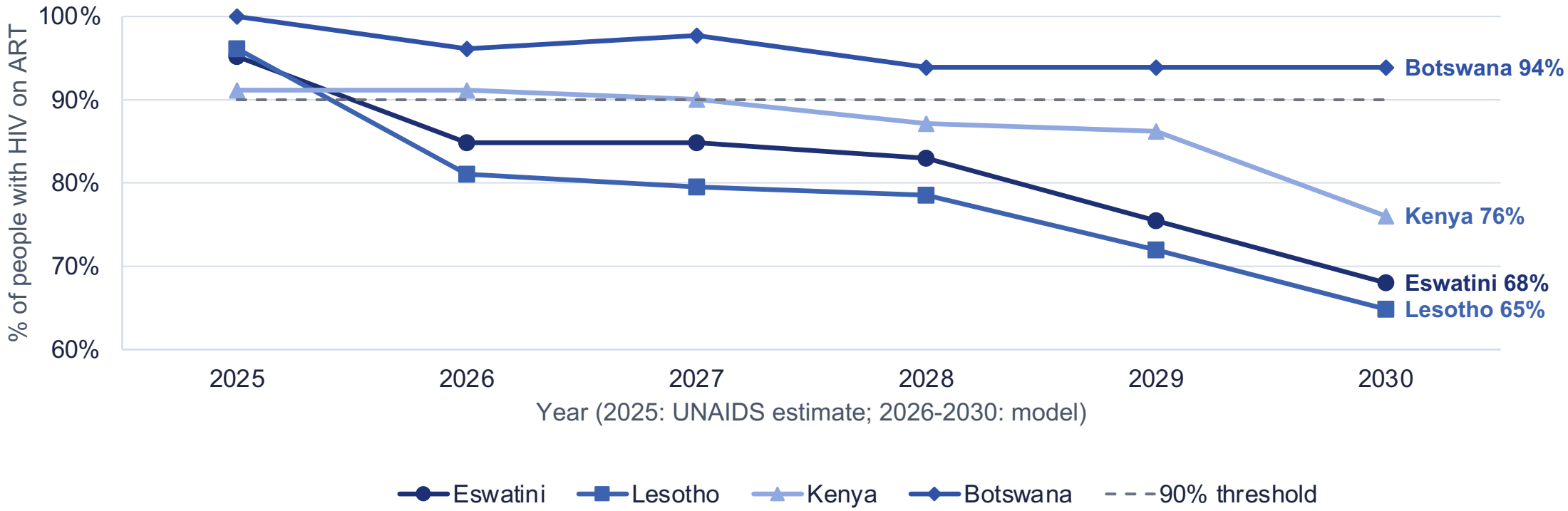
Each channel cut alone, by the share losing US$10M a year per country (half its baseline if below US$20M).
Sudden (Default) path, no replacement; full-cost equivalents after 15% continuity (assumption).
Gavi money buys vaccines only, so a Gavi cut removes no ART.

Source: model output, audit/channels/run_channels.py (health_model.run_country, central run); IHME DAH 1990-2026, 2021-23 average.

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Under the MOUs, ART coverage falls below 90% in two countries in 2026

ART coverage on signed MOU schedules, 4 of 5 MOU countries above 90% in 2025



Eswatini and Lesotho fall below 90% in 2026 and Kenya in 2028; Botswana stays at 94% or above.

US bilateral cuts on each signed MOU schedule only (no US multilateral exits); no replacement; 15% continuity (assumption).

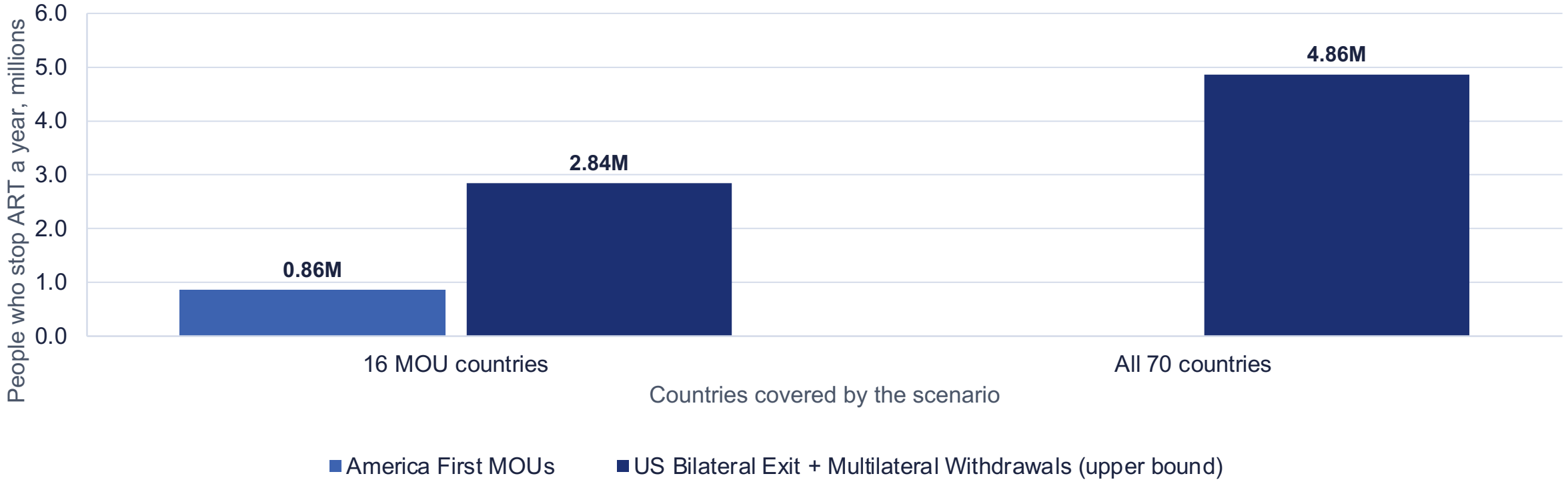
Rwanda (94% in 2025) is not shown: its result rests on an estimated count of donor-funded patients.

Source: UNAIDS AIDSinfo 2026 (2025 coverage); KFF MOU tracker and team MOU sheet; model output, audit/concentration/run_concentration.py.

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Upper bound: 4.86M people a year stop ART under a full US exit

People who stop ART a year (full-cost equivalents), 2026-30 average, by Donor Scenario



Full exit: US Bilateral & NGO aid −100% from 2026; US money via WHO, Gavi, UNFPA −100%; US Global Fund pledge −23%.
America First MOUs: US bilateral cuts on each signed MOU schedule plus the same US multilateral exits. No replacement.
Aid lost a year: US$0.70B (MOUs, 16 countries); US$1.90B (full exit, 16); US$3.96B (full exit, 70).

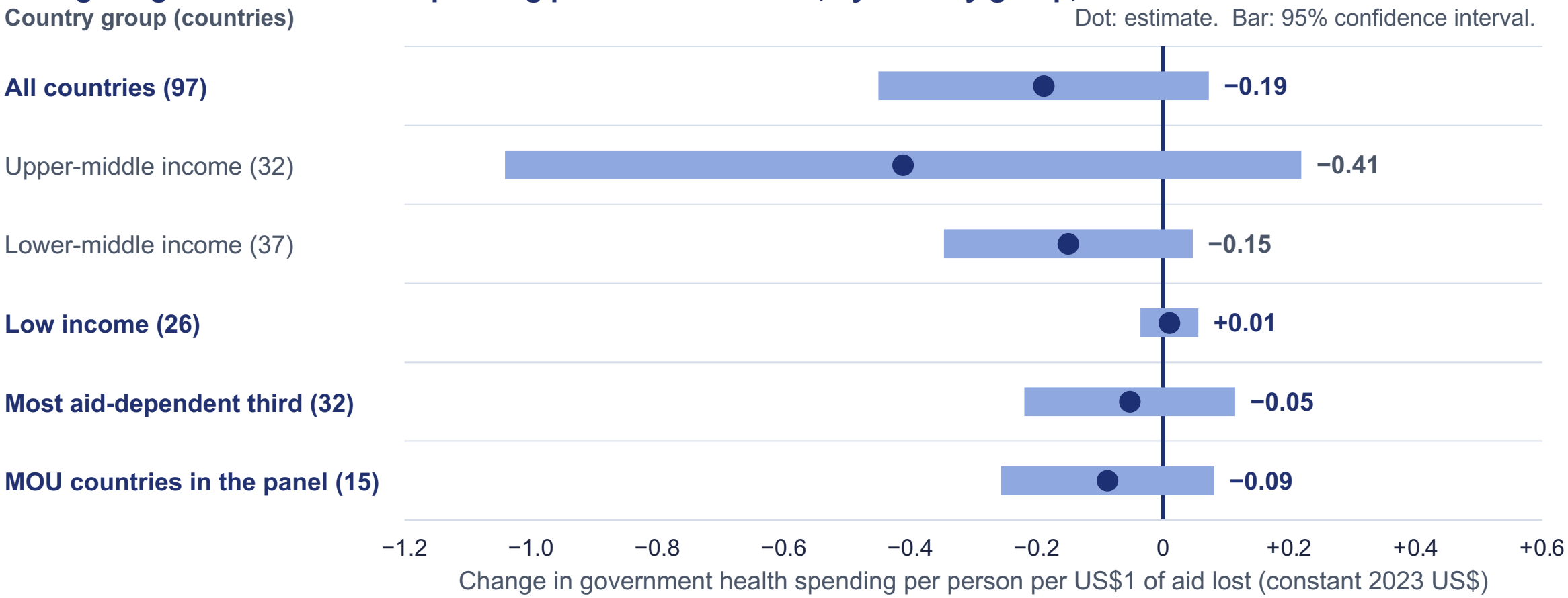
Source: model output (health_model.run_country, central run), presets America First MOUs and US Bilateral Exit + Multilateral Withdrawals; IHME DAH 2021-23.

Section D: What governments can replace

01	**Problem**	How much lost aid a government replaces decides how many people lose a service.
02	**Approach**	A two-way fixed-effects regression on IHME data, 97 countries, 2002-23; in the dashboard, Government Response sets the replacement for any Donor Scenario.
03	**Finding**	When aid fell, low-income governments neither replaced it nor cut their own health spending (+0.01 per US$1 lost, 26 countries).

When aid fell, low-income governments kept health spending unchanged

Change in government health spending per US$1 of aid lost, by country group, 2002-23



Source: IHME Health Spending 1995-2023 (GHES per person); IHME DAH 1990-2026; prepare_model.twoway_fe; audit/fiscal_history/run_fiscal_history.py.

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Section E: Insights

01	**Problem**	To guide WHO, a result has to hold across countries and donors, not rest on one country.
02	**Approach**	All six presets and six single-channel cuts run for all 70 countries; results resting on flagged inputs (TB costs, vaccine unit costs, estimated donor-funded patients) dropped.
03	**Finding**	Five results kept: US funding share, Global Fund & Gavi Shortfalls, ART per US$1M by channel, MOU ART coverage, past government response.

Five results hold across countries, donors and channels

1	The US pays 50% (US$5.2B of US$10.3B a year) of HIV, TB, malaria and immunization aid in 70 countries, and over 50% in 10 of the 20 largest recipients.
2	Global Fund & Gavi Shortfalls (every donor: Global Fund −20%, Gavi −15%) removes US$1.02B of aid a year across 70 countries.
3	Per US$1M lost, a US Bilateral & NGO cut removes 1,083 ART patient-years a year and a Global Fund cut 407 (58 and 68 countries funded).
4	On signed MOU schedules (US bilateral cuts only), ART coverage falls below 90% in Eswatini and Lesotho in 2026 and Kenya in 2028.
5	When aid fell in 2002-23, government health spending in 26 low-income countries did not move: +0.01 per US$1 lost (−0.04 to +0.06).

Sources: IHME DAH 1990-2026, 2021-23; model output by preset (audit/slide_numbers.md table b, audit/channels, audit/concentration, audit/fiscal_history).

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